

PUBLIC EDUCATION DOCUMENT · MAY 2026

WHAT DISABILITY ACTUALLY MEANS

The Conditions, Lives, and Realities Behind the Numbers

A reference-grade public education document on what cognitive, mental, physical, intellectual, congenital, and acquired disabilities actually are — what they look like, what they hinder, who they affect — and what AISH and the wider Alberta disability system does and does not provide for the 79,290 Albertans who depend on it.

**It takes one accident.
One diagnosis.**

Disability does not discriminate. It is not a moral failure, a personal weakness, or a fixed identity that belongs to other people. It is a category that any human body or brain can enter at any moment — through illness, injury, age, infection, or chance — and it is a category most of us will be in by the end of our lives whether we plan for it or not.

This document exists because public ignorance about what disability actually is enables public indifference to what disabled people actually need. Education is the precondition for change.

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The Alberta Disability System Breakdown

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WHY THIS DOCUMENT EXISTS

There is a kind of public conversation about disability that happens whenever a government cuts a program, narrows eligibility for a benefit, or moves a population from one administrative system to another. The conversation is recognizable because it follows a predictable shape. Someone on television says the disabled population should be encouraged to work. Someone on social media says they know someone on disability who is faking it. Someone in a comment section says their tax dollars should not pay for people who could pull themselves together. The arguments differ in tone but share a foundation: an assumption that the people on these programs have a clearer path to self-sufficiency than they are taking.

This document exists because that foundation is wrong. It is wrong as a matter of medical fact, social policy, demographic pattern, and human experience. The 79,290 Albertans currently receiving Assured Income for the Severely Handicapped (AISH) did not arrive there by choice. They arrived through a clinical assessment that the Government of Alberta itself confirmed, signed off on, and continues to approve every year. The conditions they live with are real, documented, and serious enough that the province's own medical reviewers determined they substantially limit the ability to earn a livelihood — the program's statutory test.

What follows is a sourced, condition-by-condition account of what the major categories of disability actually mean. What they are. What they hinder. Whether they are visible or invisible to a casual observer. How a person can come to have one. How an existing condition can change. What the AISH handbook provides as support — and what it does not. And, in closing, why every reader of this document should consider how much closer they are to needing these supports than they may currently believe.

WHO THIS DOCUMENT IS FOR

This document is written for the public — for non-disabled readers who have never had reason to learn what these conditions actually are, for elected officials who shape disability policy without lived experience of it, for journalists trying to write fairly about a community they may not know, and for anyone in the disability community who wants a single source they can hand to a friend, a family member, a coworker, or a stranger when the next conversation starts. It is a starting point, not an endpoint. It is comprehensive without claiming to be complete. The disabled experience is not reducible to a document. But understanding can begin in one.

THE NUMBERS: WHO AISH SERVES

Before any condition is described, the population must be named. The Government of Alberta publishes monthly aggregated data on the AISH caseload through its open data portal. The most recent published data (September 2025) breaks down the 79,290 active recipients by primary medical condition — that is, the condition the program identified as the substantive basis for eligibility. Many recipients have multiple conditions; the primary condition is the one recorded on their file as the lead clinical reason for AISH approval.

Primary Medical Condition	Recipients	% of Caseload
Physical Disabilities	32,901	41.5%
Mental Illness Disorders	23,940	30.2%
Cognitive Disorders	22,419	28.3%
Unknown (data entry, awaiting correction)	30	0.0%
TOTAL	79,290	100%

Source: Government of Alberta, Assisted Living and Social Services — AISH Caseload Open Data, September 2025.

Several patterns are worth naming up front:

- **Physical disabilities are the largest category** — not cognitive or mental, as is sometimes assumed. More than four in ten AISH recipients have a physical disability as their primary diagnosis. Many of these are invisible to a casual observer.
- **The three categories overlap heavily.** A person classified under one primary diagnosis may have additional conditions across the other two. Multiple/comorbid disability is the norm in the AISH caseload, not the exception.
- **86.0% of AISH recipients are single individuals.** This is not a population with intact family financial support to absorb the cost of inaccessibility. The stereotype of the disabled person living off their parents' resources does not match the demographic reality.
- **Only 16.1% have any employment income.** The remaining 83.9% have no employment income — not because they refuse to work, but because the conditions that qualified them for AISH substantially prevent it.
- **20.4% have CPP-Disability income.** Roughly one in five AISH recipients qualified for federal CPP-D, which itself requires having paid into the Canada Pension Plan and being assessed as severely and permanently disabled by federal medical reviewers. CPP-D approval is itself an independent verification of disability status.

WHAT 'PRIMARY MEDICAL CONDITION' DOES NOT MEAN

The classification 'physical disability' or 'mental illness' as a primary medical condition does not mean the person has only that condition. It means that condition was the lead clinical basis for their AISH approval. A person with severe mental illness may also have a physical disability, a cognitive impairment from medication side effects, or chronic pain. A person with a physical disability may also have depression secondary to chronic illness, anxiety, or a learning difference. The categories below describe disability at the medical-classification level. The lived reality of any individual recipient is almost always more complex than a single category.

SECTION 1: COGNITIVE DISABILITIES (28.3% / 22,419 ALBERTANS)

1.1 What Cognitive Disability Means

Cognitive disability is an umbrella term for a wide range of conditions that affect how the brain processes, stores, retrieves, and uses information. The U.S. Federal Communications Commission and the Canadian Government's Digital Accessibility Toolkit both define cognitive disability as covering conditions that include intellectual disability, autism spectrum disorder, severe and persistent mental illness with cognitive components, brain injury (traumatic or acquired), stroke, and Alzheimer's disease and other dementias.

The Web Accessibility Initiative of the W3C describes cognitive and learning disabilities as affecting how people store, retrieve, or use information. Often, only some functions are impaired while others are unaffected. Many of these disabilities do not affect overall intelligence. They are typically invisible — and many people who have them do not have a formal diagnosis or do not disclose having one due to stigma, vocational concerns, or workplace prejudice.

1.2 What Cognitive Disability Hinders

Depending on the specific condition and its severity, cognitive disability can hinder one or more of the following functions:

- **Working memory** — holding information in mind long enough to use it. Someone with a working memory impairment may forget what they were just told, lose their place mid-task, or struggle to follow multi-step instructions even seconds after hearing them.
- **Executive function** — planning, prioritizing, sequencing, initiating, and completing tasks. This includes things like opening mail, paying a bill on time, getting to an appointment on the right day, or completing a paperwork chain across multiple steps.
- **Processing speed** — how quickly the brain takes in and acts on new information. Slow processing speed makes time-pressured situations (phone trees, interviews, emergency decisions) acutely difficult.
- **Reading and comprehension** — understanding written language, especially complex or formal language. Many cognitive disabilities affect first-read comprehension; the person may need to read something three or four times to make sense of it, or may need it read aloud and explained.
- **Communication and social processing** — interpreting tone, sarcasm, social cues, eye contact, conversational rhythm. This is particularly common in autism spectrum conditions but also occurs in acquired brain injury, dementia, and severe mental illness.
- **Numerical reasoning** — calculating amounts, comparing values, managing budgets, understanding percentages. Dyscalculia is a specific cognitive condition affecting math; many other conditions impair number reasoning more broadly.
- **Sensory integration** — processing input from the environment without being overwhelmed. Sensory overload (loud noise, bright light, crowded spaces, multiple conversations) can shut down higher cognitive function entirely for some people.
- **Sustained attention** — staying focused for extended periods. Attention conditions like ADHD, but also dementia, brain injury, severe depression, and chronic pain, all impair attention in different ways.

1.3 Visible or Invisible?

Almost always invisible. Cognitive disability rarely shows on a person's face or body. The W3C explicitly notes that cognitive and learning disabilities are usually invisible. A person may struggle profoundly with form completion, sustained conversation, or memory of a phone call from yesterday — and look entirely ordinary while doing so. This invisibility is a major source of public skepticism: 'they don't look disabled.' That sentence describes the observer's failure to perceive, not the disabled person's actual capacity.

— Cognitive disability — what it can look like in a single afternoon —

Imagine sitting at the kitchen table with a piece of mail you know is important. You read the first paragraph. By the time you reach the third paragraph, the words from the first paragraph are gone. You read it again. The same thing happens. You know there is a deadline. You cannot quite hold what the deadline is for, what you need to do, and what happens if you don't, in your mind at the same time. You know your friend Tom would understand it instantly, but Tom doesn't live nearby. You also know the call centre number on the form will not be helpful — last time, by the third question, you forgot what your own follow-up was. The mail goes back in the envelope. The envelope goes on the counter. The deadline passes. You will be told later, by the program, that you were non-compliant.

1.4 The Subcategories

Within cognitive disability as the AISH program classifies it, the following are common diagnostic subcategories. Each is described below in terms of what it is, what causes it, and what it tends to hinder.

Intellectual Disability

Intellectual disability (formerly called mental retardation, a term now considered offensive and clinically retired) is defined by the American Association of Intellectual and Developmental Disabilities as significant limitations in both intellectual functioning (reasoning, learning, problem-solving — typically an IQ of 70 or lower) and adaptive behavior across everyday social and practical skills, with onset before age 18. Intellectual disability is lifelong. It does not improve with medication or treatment. It can range from mild (capable of independent living with some support) to profound (requiring total care for daily living tasks).

In Alberta, an IQ of 70 or below combined with significant adaptive skill limitations beginning before age 18 is the threshold for Persons with Developmental Disabilities (PDD) program eligibility. Many adults with intellectual disability receive both AISH (income) and PDD (services).

Autism Spectrum Disorder (ASD)

Autism is a neurodevelopmental condition affecting how a person communicates, processes social information, and experiences sensory input. It is a spectrum: support needs vary widely across individuals. Some autistic adults live independently with no formal supports; others require continuous care. Many autistic people have significant strengths — pattern recognition, deep focus, exceptional memory in areas of interest — alongside their challenges. Autism is not an illness. It is a different way the brain is wired, present from birth, lifelong.

AISH eligibility for autistic adults is based on functional impairment that substantially limits earning a livelihood. Autistic adults with significant communication, sensory, or executive function impairments often qualify; those who can perform competitive work with accommodations typically do not.

Acquired Brain Injury (ABI) / Traumatic Brain Injury (TBI)

Acquired brain injury covers any damage to the brain that occurs after birth — including traumatic injury (a fall, a car accident, an assault, a sports concussion), stroke, brain hemorrhage, infection, lack of oxygen, and substance-related damage. The Brain Injury Association of America notes that brain injury is often called an 'invisible disability' because while some effects (motor coordination, speech) are visible, many others (memory, executive function, emotional regulation, fatigue) are not.

Brain injury is one of the strongest demonstrations of the document's closing argument: a single event — a single fall, a single accident, a single moment of oxygen loss — can produce permanent cognitive disability in a person who was, the day before, fully able-bodied.

Dementia and Mild Cognitive Impairment

Dementia is a progressive loss of cognitive ability beyond what is expected from normal aging. It includes Alzheimer's disease, Lewy body dementia, frontotemporal dementia, vascular dementia, and dementia related to Parkinson's disease and Huntington's disease. In early stages, often called mild cognitive impairment (MCI), the person may experience forgetfulness, word-finding difficulty, and trouble concentrating. In later stages, the person becomes incapable of independent living due to the inability to plan, sequence, and apply judgment to daily tasks.

While most dementia is associated with aging, early-onset dementia (before age 65) does occur and can affect AISH-aged recipients. Some dementias have genetic components; others develop after brain injury, stroke, or substance exposure.

Learning Disabilities (Dyslexia, Dyscalculia, Dysgraphia)

Learning disabilities affect specific cognitive domains while leaving overall intelligence intact. Dyslexia affects reading. Dyscalculia affects math. Dysgraphia affects writing. These conditions can be congenital, hereditary, or acquired through brain injury or aging. Severity ranges from mild (manageable with accommodation) to profound. Adults with severe learning disabilities may be functionally illiterate or unable to manage finances despite normal or above-average intelligence.

ADHD (Attention-Deficit / Hyperactivity Disorder)

ADHD is a neurodevelopmental condition affecting attention regulation, impulse control, and executive function. It is not a deficit of attention but a difficulty regulating where attention goes — which is why people with ADHD can hyperfocus on some things while struggling to start others. Severe ADHD can substantially impair employment, financial management, and consistent task completion. Like autism, ADHD is lifelong; treatment may include medication, therapy, and accommodation, but the underlying neurodevelopmental pattern persists.

1.5 Visible vs Invisible Within Cognitive Disability

Condition	Visibility	What May Be Visible / What Is Hidden
Intellectual disability (severe)	Sometimes visible	Communication patterns may be apparent. Reasoning and adaptive limitations are hidden.
Intellectual disability (mild)	Usually invisible	Most people with mild intellectual disability look and sound typical in casual conversation.

Condition	Visibility	What May Be Visible / What Is Hidden
Autism spectrum disorder	Variable	Some autistic people have visible movement or communication patterns. Many do not.
Acquired brain injury	Mixed	Motor effects may be visible. Memory, fatigue, emotional regulation are hidden.
Dementia (early)	Almost always invisible	Memory and word-finding problems do not show on appearance.
Dementia (advanced)	Often visible	Confusion, disorientation, and care needs become apparent.
Learning disabilities	Almost always invisible	Reading, math, and writing impairments are not visible until the person attempts the task.
ADHD	Almost always invisible	Behavioural patterns may be perceived; underlying executive function impairment is hidden.

SECTION 2: MENTAL ILLNESS DISORDERS (30.2% / 23,940 ALBERTANS)

2.1 What Mental Illness Means

Mental illness disorders are conditions that affect a person's mood, thinking, perception, behaviour, or some combination of these. The Centre for Addiction and Mental Health (CAMH), Canada's largest mental health and addiction teaching hospital, defines mental illness as conditions that significantly impair a person's ability to function in work, relationships, and daily life. Mental illness covers a broad spectrum, from mild and time-limited episodes to severe, chronic, and life-altering conditions.

The Statistics Canada Canadian Survey on Disability (2017) defines persons with a mental health-related disability as those who experience limitations in their daily activities because of difficulties with an emotional, psychological, or mental health condition. Examples explicitly include anxiety, depression, bipolar disorder, schizophrenia, eating disorders, and substance use disorders.

Crucially: the AISH program's classification of mental illness as a primary medical condition does not refer to a passing episode of distress. It refers to chronic, severe, and impairing mental illness — the kind that has been documented by clinicians, has resisted standard treatment, and has substantially prevented the person from earning a livelihood.

2.2 What Mental Illness Hinders

Severe mental illness can hinder a remarkable range of functions, depending on the specific condition:

- **Concentration and cognitive function** — major depression, severe anxiety, and active psychosis all interfere with the brain's ability to focus, plan, remember, and decide. This is sometimes called 'brain fog' but the term understates it. Severe mental illness can produce cognitive impairment indistinguishable, on testing, from mild dementia.
- **Sleep regulation** — almost all severe mental illness affects sleep, either by causing insomnia (anxiety, mania) or hypersomnia (depression). Disrupted sleep then compounds every other symptom.
- **Emotional regulation** — the ability to feel an emotion, act on it appropriately, and let it pass. Severe mental illness can cause emotions to flatten (depression), intensify (anxiety, mania), invert (depression masking as irritability), or fragment from coherent self-experience entirely (dissociation, psychosis).
- **Reality testing** — in psychotic conditions, the basic distinction between internal experience and external reality can break down. The person may experience hallucinations, hold delusional beliefs, or be unable to evaluate the accuracy of their own perceptions.
- **Energy and motivation** — severe depression often presents not as sadness but as profound fatigue and inability to initiate any action, including the actions needed to seek help. The person may be unable to shower, eat, or leave the house, while looking, to outsiders, simply lazy.
- **Social functioning** — anxiety, paranoia, dissociation, and emotional flooding all interfere with the basic exchanges of daily life. Phone calls become impossible. Doctor's appointments become impossible. Grocery store visits become impossible.
- **Self-preservation** — severe mental illness is among the leading causes of premature death in Canada. CAMH documents that mental illness and substance use disorders are leading causes of disability in Canada and that people with these conditions are more likely to die prematurely than the general population. Suicide

risk is a real and ongoing dimension of severe illness.

2.3 Visible or Invisible?

Almost always invisible. This is a defining feature of mental illness as a disability category. A person can be in profound psychological distress, hallucinating, suicidally depressed, or actively manic, and look — to a casual observer — entirely ordinary. The Government of Canada's accessibility guidance specifically notes that mental health disabilities affect mood, energy, concentration, and physical health, and that many are invisible or hidden.

This invisibility is a particular feature of public skepticism about mental illness as a 'real' disability. The skepticism is misplaced. The 2020 Disability Adjusted Life Years (DALY) data from the Institute for Health Metrics and Evaluation indicate that depressive, anxiety, bipolar, and psychotic disorders together accounted for over 10% of total disease burden in Canada. In some jurisdictions, the burden of major depressive disorder alone exceeds the combined burden of breast, colorectal, lung, and prostate cancers.

— Mental illness — what it can look like on a Tuesday —

Imagine waking up in a body that has been awake all night, but a brain that is moving through molasses. Your alarm went off three hours ago. You are still in bed. You can think clearly enough to know that getting up would be a good idea. You cannot, with that same clarity, locate the part of yourself that initiates getting up. Your therapist would call this 'volitional impairment'. You would call it: I cannot move. By eleven, you have eaten nothing. The bathroom feels far. The phone has six unread messages and you cannot read them because reading them would require you to do something with the information, and you cannot do anything with information today. Your friends, who love you, ask why you cancelled. You cannot answer because the answer is: I could not generate the speech-act required to cancel. You looked, on the outside, perfectly fine. On the inside, you fought a war against a brain that was not letting you out of bed. You won the war by making it to the bathroom by 2 PM. That counts as a victory. No one watching would have called it one.

2.4 The Subcategories

Major Depression and Mood Disorders

Over 11 percent of Canadians have a mood disorder diagnosis (Statista, 2024). Major depressive disorder produces persistent low mood, loss of interest, fatigue, sleep and appetite disturbance, and difficulty concentrating. Severe major depression is associated with suicidal ideation, psychotic features, and complete inability to function in daily life. Some major depression is treatment-resistant — meaning standard medications and therapies do not produce improvement. Treatment-resistant depression is a recognized basis for AISH eligibility.

Anxiety Disorders

Anxiety disorders include generalized anxiety disorder, panic disorder, social anxiety disorder, and specific phobias. Severe anxiety is not 'feeling worried.' It can include panic attacks (sudden episodes of acute physical and psychological distress), agoraphobia (inability to leave the home), and persistent autonomic activation that produces real cardiac, gastrointestinal, and immune effects. Severe anxiety can substantially limit employment,

social functioning, and access to healthcare itself.

Bipolar Disorder

Bipolar disorder is characterized by alternating episodes of depression and mania (or hypomania). Mania is not happiness — it is a state of impaired judgment, racing thoughts, decreased need for sleep, impulsive behaviour, and sometimes psychosis. People with severe bipolar disorder can lose savings, jobs, marriages, and housing during manic episodes, and then enter depressive episodes that prevent them from rebuilding. Bipolar disorder more severely affects young people and is the sixth-leading cause of disability-adjusted life years among people aged 10 to 24 worldwide.

Schizophrenia and Psychotic Disorders

Schizophrenia is a chronic mental disorder affecting thinking, perception, and behaviour. It typically involves positive symptoms (hallucinations, delusions, disorganized speech) and negative symptoms (flattened affect, social withdrawal, reduced motivation, cognitive impairment). Schizophrenia substantially impairs employment, relationships, and self-care. At least 20% of people with mental illness have a co-occurring substance use disorder; for people with schizophrenia, the figure rises to as high as 50%, often as a form of self-medication for unrelieved symptoms.

Post-Traumatic Stress Disorder (PTSD)

PTSD develops after exposure to traumatic events — combat, assault, severe accident, child abuse, witnessed death, or persistent traumatic stress. Symptoms include intrusive memories, flashbacks, nightmares, hypervigilance, emotional numbness, and avoidance. PTSD is recognized in Canada as a disability when it significantly impairs daily activities and work. Veterans Affairs Canada provides specific disability benefits to veterans with service-related PTSD.

Personality Disorders

Personality disorders are enduring patterns of inner experience and behaviour that deviate markedly from the expectations of the person's culture. Borderline personality disorder, for instance, is characterized by emotional dysregulation, identity disturbance, unstable relationships, and self-harming behaviour. Severe personality disorder can substantially impair employment, relationships, and self-care. Treatment is intensive and long-term; some forms are partially treatable, others are largely untreated by current medicine.

Eating Disorders

Severe eating disorders — including anorexia nervosa, bulimia nervosa, and binge eating disorder — are mental illnesses with substantial physical consequences. Anorexia has the highest mortality rate of any mental illness. Severe and persistent eating disorders are recognized bases for disability benefits in Canada when they substantially impair daily functioning.

Substance Use Disorders

Substance use disorders are recognized mental health conditions and frequently co-occur with other mental illness. They are not lifestyle choices: substance use disorders involve neurochemical changes in the brain that produce compulsive use despite consequences. Treatment is medical, intensive, and often requires specialized support. People with substance use disorders alongside other mental illness are among the most underserved populations in Canadian healthcare.

ON SEVERITY

Mental illness exists on a spectrum from mild and time-limited to severe and lifelong. The 23,940 Albertans on AISH with mental illness as their primary diagnosis are not people experiencing situational stress or temporary low mood. They are people whose mental illness has been clinically documented, has resisted treatment, and substantially prevents employment. The popular framing of mental illness as something to 'push through' or 'be resilient about' applies — at most — to mild and transient forms. It does not apply to the severe, chronic illness this category refers to.

SECTION 3: PHYSICAL DISABILITIES (41.5% / 32,901 ALBERTANS)

This is the largest single category of AISH primary diagnoses. It is also the most popularly misunderstood, because public imagination of physical disability defaults to mobility aids — wheelchairs, walkers, prosthetics — while the actual category includes vast numbers of conditions that produce no visible signs whatsoever.

3.1 What Physical Disability Means

Physical disability covers conditions that affect mobility, dexterity, stamina, sensory function, internal organ function, or the body's underlying structural and metabolic systems. The Government of Canada accessibility guidance explicitly notes that 'many mobility, flexibility, or dexterity disabilities are invisible or hidden,' and that 'in some cases, a person's ability to move may depend on how long they need to stand or how far they need to walk.'

Physical disability includes both stable, lifelong conditions (cerebral palsy, congenital limb difference, spinal cord injury sustained in childhood) and progressive, fluctuating, or episodic conditions (multiple sclerosis, ALS, lupus, fibromyalgia, chronic pain). The visible/invisible distinction is more important here than in any other category, because so much of the public conversation treats 'physical disability' as synonymous with 'visible disability.'

3.2 Visible Physical Disabilities

Mobility Impairments

Mobility impairments include conditions affecting the legs, hips, spine, or central nervous system that limit walking, standing, climbing, or transferring. Causes include spinal cord injury, cerebral palsy, muscular dystrophy, post-polio syndrome, severe arthritis, amputation, congenital limb difference, and post-stroke hemiplegia. Mobility impairment may require wheelchairs, walkers, scooters, canes, prosthetics, or orthotics — though many people with significant mobility impairment do not use visible aids in all settings.

Sensory Impairments — Blindness and Low Vision

Blindness and low vision range from total absence of light perception to functional blindness with some residual vision. Causes include congenital conditions (retinitis pigmentosa, congenital cataract, optic nerve hypoplasia), acquired conditions (macular degeneration, diabetic retinopathy, glaucoma, retinal detachment), and traumatic injury. People with severe vision loss may use white canes, guide dogs, screen readers, or braille; many use no visible aids in familiar environments.

Sensory Impairments — Deafness and Hearing Loss

Deafness and hearing loss range from mild to profound. Some Deaf people identify as members of a linguistic and cultural community using ASL or LSQ; others use hearing aids, cochlear implants, or assistive listening devices; others communicate primarily through written language and lip reading. Hearing loss is largely invisible, especially when the person uses signing or text-based communication.

3.3 Invisible Physical Disabilities

These are the conditions that surprise people. They produce real, severe, persistent disability — and produce no visible sign of it.

Chronic Pain

The Canadian Pain Society defines chronic pain as pain lasting longer than three months. Approximately 8 million Canadians live with chronic pain — roughly one in five Canadian adults. Causes include arthritis, fibromyalgia, neuropathy, post-surgical pain, post-injury pain, autoimmune disease, and pain with no identifiable structural cause (idiopathic pain). Chronic pain is largely invisible, frequently disbelieved, and one of the most underserved disability categories in Canadian healthcare.

The Government of Canada's Action Plan for Pain in Canada explicitly recognizes chronic pain as a disability and emphasizes the importance of integrating it into broader health and disability systems.

Fibromyalgia

Fibromyalgia is a chronic condition characterized by widespread musculoskeletal pain, fatigue, sleep disturbance, cognitive difficulties (often called 'fibro fog'), and heightened sensitivity to pressure and touch. Between 2% and 8% of Canadians are affected, with women disproportionately impacted. Over 50% of people with fibromyalgia do not have paid employment, and those who do lose an average of 22.4 working days per year because of their condition.

Fibromyalgia does not show on x-rays, MRIs, or blood tests. This is the source of much of the historical disbelief surrounding the condition. Canadian courts and insurance companies acknowledge fibromyalgia as a disabling condition, but disability claims are routinely denied at first application due to the lack of objective imaging or laboratory findings.

Multiple Sclerosis (MS)

Multiple sclerosis is an autoimmune condition in which the body's immune system attacks the protective covering of nerve fibres in the brain and spinal cord. MS produces a wide range of symptoms — fatigue, weakness, vision problems, cognitive impairment, balance problems, pain, and bladder dysfunction. Canada has one of the highest MS rates in the world. MS can be relapsing-remitting (episodes followed by recovery), secondary progressive (relapsing-remitting that becomes steadily worsening), or primary progressive (steadily worsening from onset). Many people with MS look entirely well during periods of remission and severely disabled during relapses.

Myalgic Encephalomyelitis / Chronic Fatigue Syndrome (ME/CFS)

ME/CFS is a chronic, complex, multi-system disease. The defining feature is post-exertional malaise — a worsening of symptoms after physical or mental exertion that can last days, weeks, or longer. Severe ME/CFS can leave people housebound or bedbound for years. ME/CFS is largely invisible, frequently disbelieved, and historically dismissed as psychosomatic. Recent research has confirmed substantial physiological abnormalities in immune, metabolic, and autonomic function. Long COVID has produced an estimated millions of new ME/CFS-like cases worldwide since 2020.

Ehlers-Danlos Syndromes (EDS)

Ehlers-Danlos syndromes are a group of inherited connective tissue disorders affecting collagen, the protein that provides structure to skin, joints, blood vessels, and organs. Hypermobile EDS is the most common form. Symptoms include chronic joint pain, frequent dislocations, easy bruising, gastrointestinal problems, and cardiovascular complications. EDS is often misdiagnosed for years before correct identification. The condition is largely invisible until symptoms manifest.

Lupus and Autoimmune Disease

Systemic lupus erythematosus is an autoimmune disease in which the body's immune system attacks its own tissues, producing widespread inflammation. Symptoms include joint pain, fatigue, skin rashes, kidney problems, cardiovascular complications, and cognitive impairment. Lupus disproportionately affects women, particularly women of colour. Like MS, lupus can fluctuate dramatically — periods of remission can give way to severe flares. Other autoimmune diseases — rheumatoid arthritis, ankylosing spondylitis, psoriatic arthritis, Sjögren's syndrome — produce similar fluctuating, often invisible disability.

Cardiovascular and Pulmonary Disease

Severe heart disease, congestive heart failure, chronic obstructive pulmonary disease (COPD), pulmonary hypertension, and other cardiovascular and respiratory conditions can substantially limit physical capacity without producing any visible sign. A person with severe heart failure may walk into a room looking ordinary and be unable to climb a flight of stairs without rest. Many AISH recipients with cardiovascular or pulmonary disease as their primary condition are entirely invisible to a casual observer.

Diabetes and Metabolic Disease

Severe diabetes — particularly with complications such as neuropathy, retinopathy, nephropathy, or unstable blood sugar — can be a substantially limiting condition. End-stage diabetic complications (kidney failure requiring dialysis, vision loss, amputation) produce both visible and invisible disability. Type 1 diabetes alone, even when well-managed, requires constant attention to blood sugar, insulin, food, exercise, and emergency response — a 24/7 medical management burden invisible to outside observers.

Cancer and Cancer Survivorship

Active cancer treatment and post-treatment survivorship can produce both visible and invisible disability. Chemotherapy and radiation can cause persistent fatigue, cognitive impairment ('chemo brain'), neuropathy, and organ damage that persists years after treatment ends. Some cancers (and their treatments) cause permanent disability through nerve damage, organ removal, or hormonal disruption. AISH eligibility for cancer-related disability typically focuses on persistent post-treatment functional impairment, not active treatment alone.

— Invisible physical disability — what it can look like on a Saturday —

Imagine standing in a grocery store. To anyone watching, you are a thirty-eight-year-old woman in jeans and a sweater, looking at a shelf of pasta. What no one watching can see: you have not eaten today because nausea has been hitting you in waves since Tuesday. The fluorescent light is making your head feel like it is being slowly compressed in a vise. You walked the seventy steps from your car to this aisle and your heart rate is now 142 beats per minute. The cane your specialist recommended is in your trunk because you are tired of the way people look at thirty-eight-year-old women with canes. Your prescription is supposed to control the autoimmune flare, but the prescription has not been filled because the pharmacy needs a renewal from your specialist, and your specialist's office no longer accepts phone messages, only an online portal that requires a password you cannot remember and cannot reset because the reset email goes to an address you no longer have access to. You will pay for groceries with the credit card because AISH was reduced this month for reasons no one has explained. You will look, to the cashier, entirely fine. None of what is wrong is visible. All of it is real.

3.4 Visible vs Invisible Within Physical Disability

Condition Category	Visibility	What May Be Visible / What Is Hidden
Wheelchair-using mobility impairment	Visible	Wheelchair, transfers. Pain, fatigue, secondary conditions are hidden.
Cane / walker mobility impairment	Often visible	Mobility aid is visible. Pain, balance, fatigue are hidden.
Spinal cord injury (with intact upper body)	Often partly visible	Wheelchair use visible. Bladder, sexual, autonomic dysfunction hidden.
Cerebral palsy (mild to moderate)	Variable	Movement patterns may be visible. Pain, fatigue are hidden.
Blindness / low vision	Sometimes visible	White cane, guide dog visible. Functional vision varies enormously.
Deafness / hearing loss	Almost always invisible	Signing or hearing aids may be perceived; functional hearing is hidden.
Chronic pain	Almost always invisible	Mobility limitations on bad days may be visible. Pain itself is not.
Fibromyalgia	Almost always invisible	Body looks unremarkable. Pain, fatigue, fog are entirely internal.
Multiple sclerosis (in remission)	Often invisible	Person may appear well between relapses. Fatigue, cognitive impact hidden.
ME/CFS	Almost always invisible	Rest is the symptom. Most people with severe ME/CFS are simply absent.
Lupus / autoimmune disease	Variable	Skin rashes may be visible. Pain, fatigue, organ involvement hidden.
Heart failure / COPD	Almost always invisible	Body looks ordinary. Cardiovascular and respiratory limits are entirely internal.
Diabetes (severe complications)	Variable	Amputation, retinopathy, dialysis access can be visible. Daily management invisible.
Cancer / cancer survivorship	Variable	Active treatment may be visible. Long-term effects are usually invisible.

WHY THIS MATTERS

Public skepticism about disability — the 'they don't look disabled' reflex — is most often deployed against people with invisible physical disability. The reflex assumes the absence of visible signs is evidence of absent disability. The data above demonstrates the reverse: most AISH recipients with physical disability as their primary diagnosis have an invisible or partially-invisible condition. Believing only visible disability is real means disbelieving most of the disabled population.

SECTION 4: INTELLECTUAL AND DEVELOPMENTAL DISABILITIES

Intellectual and developmental disabilities (IDD) are a subset of cognitive disability defined by onset before age 18 and substantial limitations in both intellectual functioning and adaptive behaviour. Statistics Canada (2022) reported that 1.5% of Canadians aged 15 and over — approximately 456,630 individuals — have a developmental disability.

In Alberta, the Persons with Developmental Disabilities (PDD) program serves adults 18 and older with developmental disabilities. PDD eligibility requires a developmental disability with onset before age 18, an IQ of 70 or below, and significant adaptive skill limitations affecting six or more of 24 daily living skills (which include reading, writing, finding and keeping a job, using emergency services, and others). Many PDD-eligible adults also receive AISH financial support; the two programs work together.

4.1 Down Syndrome

Down syndrome (also called Trisomy 21) is a genetic condition caused by an extra copy of chromosome 21. It is the most common chromosomal condition diagnosed in Canada. People with Down syndrome have characteristic physical features, intellectual disability ranging from mild to moderate (occasionally severe), and elevated risk for certain medical conditions including congenital heart defects, hearing problems, vision problems, thyroid disease, and early-onset Alzheimer's disease.

People with Down syndrome have a wide range of capabilities. Many live with substantial independence with appropriate support; some require continuous care for daily living. Adults with Down syndrome are common participants in PDD programs and AISH. The Statistics Canada Canadian Survey on Disability identifies a respondent who has been diagnosed with a developmental disorder such as Down syndrome as having a disability regardless of activity limitation level — recognizing that the diagnosis itself implies functional support needs.

4.2 Fetal Alcohol Spectrum Disorder (FASD)

Fetal Alcohol Spectrum Disorder is a permanent neurodevelopmental disability caused by prenatal alcohol exposure. FASD affects brain development and produces lifelong cognitive, behavioural, and physical challenges. The condition is invisible in the sense that most people with FASD do not have the physical features once associated with the term 'fetal alcohol syndrome' — but the underlying brain differences are real, permanent, and substantially impairing.

FASD is one of the leading causes of developmental disability in Canada. People with FASD often have difficulty with executive function, impulse control, working memory, social judgment, and adaptive behaviour. Without appropriate supports, adults with FASD are over-represented in the criminal justice system, the homeless population, and in disability income support programs. With appropriate supports, many adults with FASD live independently.

4.3 Cerebral Palsy

Cerebral palsy is a group of permanent movement disorders caused by damage to the developing brain, usually before, during, or shortly after birth. The condition is non-progressive — the underlying brain injury does not

worsen — but its physical effects can change over time, particularly in adulthood, where joint problems, pain, and fatigue may become significant.

Cerebral palsy ranges from mild (slight motor impairment with full intellectual capacity) to severe (no independent movement, no verbal communication, full physical dependence). Many people with cerebral palsy have normal or above-average intelligence and significant physical disability; others have intellectual disability alongside the motor condition. Cerebral palsy is one of the most heterogeneous categories of disability — no two people with the same diagnosis present identically.

4.4 Tourette Syndrome

Tourette syndrome is a neurodevelopmental condition characterized by motor and vocal tics — sudden, repetitive, non-rhythmic movements or vocalizations. Tics range from mild and barely noticeable to severe and physically disruptive. Some people with Tourette have coprolalia (involuntary swearing), though this affects a minority of people with the condition. Tourette commonly co-occurs with ADHD, OCD, anxiety, and learning disabilities.

4.5 Prader-Willi Syndrome

Prader-Willi syndrome is a rare genetic condition affecting chromosome 15. It produces intellectual disability, behavioural challenges, and a defining feature of insatiable hunger that, without strict food management, leads to life-threatening obesity. Adults with Prader-Willi typically require lifelong supervised living arrangements with controlled food access.

4.6 Other Genetic and Chromosomal Conditions

Many other genetic and chromosomal conditions produce intellectual and developmental disability. Fragile X syndrome is the most common inherited cause of intellectual disability. Williams syndrome, Angelman syndrome, Cri du chat syndrome, Smith-Magenis syndrome, and many others each produce distinctive patterns of intellectual, physical, and behavioural features. Together, these conditions represent thousands of Albertans navigating PDD, AISH, and the broader disability system.

— Developmental disability — what it can look like at age twenty-two —

Imagine being twenty-two years old and living with your parents because the program that was supposed to provide you with supported housing has a waitlist that has not moved in three years. You finished high school, and the school system, which had supported you reasonably well, no longer applies. The PDD program assigned you a worker, who was caring, but who left after six months and was not replaced for another four. You like to work — you had a part-time job at a grocery store that you loved — but the job ended when your hours were cut for reasons no one explained, and finding a new one has been impossible because the postings all assume online application skills you do not have. You receive AISH and PDD. Your parents are in their sixties. Both your parents have started to talk about who will take care of you when they cannot. There is no clear answer. There is only a waitlist.

SECTION 5: BIRTH DEFECTS AND CONGENITAL CONDITIONS

Congenital conditions — conditions present at birth — represent a significant portion of lifelong disability. Some are genetic; some result from in-utero environmental exposure; some have unknown causes. Public Health Agency of Canada data indicates that approximately 3% of all live births in Canada involve some form of congenital anomaly. Many of these are minor and do not produce lasting disability; others produce substantial lifelong disability.

5.1 Spina Bifida

Spina bifida is a neural tube defect in which the spine does not close completely during fetal development. Severity ranges from mild (no functional impact) to severe (paralysis below the level of the defect, hydrocephalus, bowel and bladder dysfunction). Severe spina bifida produces lifelong physical disability requiring mobility aids, medical management, and often surgical intervention. People with severe spina bifida who survive infancy often live full lives with appropriate support.

5.2 Congenital Heart Defects

Congenital heart defects are structural problems with the heart present at birth. Some are minor and self-resolving; others require surgical repair in infancy and lifelong cardiac monitoring. Severe congenital heart defects can substantially limit physical capacity, produce chronic fatigue, and require multiple surgeries across the lifespan. Adults living with surgically-corrected congenital heart disease may appear healthy while having significant cardiac limitations invisible to outside observers.

5.3 Cystic Fibrosis

Cystic fibrosis is a genetic condition primarily affecting the lungs and digestive system. Thick mucus accumulation in the lungs produces persistent infection and progressive lung damage. Treatment is intensive and lifelong, including daily airway clearance, multiple medications, and pancreatic enzyme replacement. While medical advances have substantially extended life expectancy for people with CF, the condition still requires hours of daily medical management and produces progressive respiratory disability over time.

5.4 Muscular Dystrophy

The muscular dystrophies are a group of genetic conditions causing progressive muscle weakness and degeneration. Duchenne muscular dystrophy, the most common severe form, typically presents in early childhood and produces progressive disability through adolescence and into adulthood. Other forms (Becker, limb-girdle, facioscapulohumeral) have variable onset and progression. Muscular dystrophy is progressive — the disability worsens over time — making lifelong support requirements predictable but increasing.

5.5 Congenital Limb Differences

Congenital limb differences include amelia (absence of a limb), meromelia (partial absence), and conditions affecting limb formation, length, or function. Causes include genetic conditions, in-utero environmental exposure, and unknown factors. Many people with limb differences live with full participation in society using prosthetics, adaptive equipment, or no aids at all. Visibility varies. The category illustrates that disability presents at birth in forms that are often visible but not necessarily disabling in every functional sense.

5.6 Hearing and Vision Conditions Present at Birth

Many forms of deafness, blindness, and low vision have congenital or early-childhood onset. Causes include genetic conditions, prenatal infection (rubella, cytomegalovirus), birth complications, and metabolic disorders. People who are deaf or blind from birth often develop language and identity around their disability in distinctive ways — Deaf culture and ASL/LSQ communities, for example, are not framed as deficits but as communities with their own languages and traditions.

— Congenital cardiac disease — what it can look like at age thirty-eight —

Imagine being born with a heart that needed two surgeries before your second birthday and a third when you started kindergarten. By the time you were twenty, you had a pacemaker. By the time you were thirty, your cardiologist explained that the surgical repairs from your childhood were starting to show wear, and that another open-heart surgery would likely be needed in the next decade. You look fine. You went to university, got a degree, started a career. The career ended when you spent six weeks in cardiac rehabilitation after an arrhythmia episode and your employer decided your medical needs were 'too disruptive.' You applied for AISH because you cannot work full-time anymore and the cost of cardiac medications, specialist appointments, and the equipment you need at home exceeds what you can earn part-time. You did not choose any of this. Your heart, before you ever drew a breath, was not formed correctly. Everything since has been a matter of medical management and the slow accumulation of consequences from a condition that was never your fault.

SECTION 6: ACQUIRED CONDITIONS — THE 'ONE EVENT' CATEGORY

This is the section that most directly demonstrates the closing argument of this document. Acquired conditions are conditions a person did not have, then had — often through a single event or a single diagnosis. They are the disabilities that prove disability is not a fixed identity. It is a category any human can enter at any time.

6.1 Traumatic Brain Injury (TBI)

Traumatic brain injury is brain damage caused by external force — a fall, a vehicle collision, an assault, a sports injury, an explosion. Severity ranges from mild concussion (typically resolving within weeks) to severe (producing permanent cognitive, physical, emotional, and behavioural changes). Brain Injury Canada estimates that approximately 165,000 Canadians sustain a brain injury each year, the majority recovering fully, but tens of thousands left with permanent disability.

A single car accident at age twenty-five can produce a brain that, at age thirty, cannot reliably hold information, regulate emotions, or sustain attention. The person before the accident is gone; the person after the accident is, in many ways, a different person inhabiting the same body. AISH recipients with brain injury as their primary condition often describe this discontinuity.

6.2 Stroke

Stroke occurs when blood flow to part of the brain is interrupted (ischemic stroke) or when a blood vessel in the brain ruptures (hemorrhagic stroke). The consequences depend on the location and extent of the brain damage: paralysis on one side of the body (hemiplegia), language impairment (aphasia), memory loss, vision problems, swallowing problems, emotional dysregulation, fatigue. While stroke risk increases with age, strokes occur in younger adults — particularly those with heart conditions, blood disorders, or other risk factors.

A stroke at age forty-eight can produce a left hemiplegia that requires daily wheelchair use, an aphasia that prevents reliable verbal communication, and a fatigue that makes a four-hour workday impossible. None of these conditions existed twenty-four hours before the event.

6.3 Spinal Cord Injury

Spinal cord injury results from damage to the spinal cord through trauma (vehicle collisions, falls, sports injuries, gunshot wounds) or disease (tumour, infection, vascular damage). Depending on the level and completeness of the injury, the person may experience paraplegia (paralysis affecting the lower body), tetraplegia (paralysis affecting all four limbs), or partial impairment. Spinal cord injury also affects bowel, bladder, sexual function, autonomic regulation, and pain — not just movement.

The Canadian Spinal Research Organization estimates approximately 86,000 Canadians live with spinal cord injury, with about 4,300 new injuries each year. The most common cause in younger adults is motor vehicle collisions; in older adults, falls. A single fall down a flight of stairs can produce permanent paraplegia.

6.4 Late-Onset Multiple Sclerosis, ALS, Parkinson's Disease

Some neurological conditions can present at any age but commonly emerge in adulthood. Multiple sclerosis most often presents between ages 20 and 50. Amyotrophic lateral sclerosis (ALS, also called Lou Gehrig's disease)

most often presents between 40 and 70 and produces progressive motor neuron death — typically resulting in death within 2 to 5 years of diagnosis, though some people live much longer. Parkinson's disease typically presents after age 60 but can present earlier (young-onset Parkinson's).

These conditions illustrate that even the apparently 'healthy' middle-aged adult is one diagnosis away from progressive neurodegenerative disability. The person who runs the marathon at 45 may be using a walker at 52. The body does not announce these conditions in advance. They begin, often, with a tremor, a fall, a moment of unexplained weakness — and then progress.

6.5 Cancer and Cancer-Related Disability

Cancer affects approximately 1 in 2 Canadians during their lifetime (Canadian Cancer Society). Active treatment produces fatigue, immunosuppression, cognitive impairment, and physical limitations. Long-term survivorship can produce permanent disability through nerve damage from chemotherapy, organ damage from radiation, hormonal disruption from treatment, and post-surgical functional limitations. Some cancer survivors qualify for AISH on the basis of post-treatment chronic disability.

6.6 Long COVID and Post-Viral Conditions

Long COVID is a multi-system condition that develops in some people after acute SARS-CoV-2 infection. Symptoms can include profound fatigue, post-exertional malaise (worsening after exertion, similar to ME/CFS), cognitive impairment ('brain fog'), cardiovascular abnormalities, autonomic dysfunction, and chronic pain. Statistics Canada and Public Health Agency data estimates that millions of Canadians have experienced long COVID symptoms; an unknown but substantial number have been left with persistent disability.

Long COVID is the clearest contemporary demonstration of acquired disability. Millions of people who were healthy in 2019 are now disabled. They did not choose this. They did not fail to be careful in some way that justifies their condition. They caught a virus, like billions of other people, and unlike most of the others, did not fully recover. They are now navigating the same disability systems described in this document.

6.7 Workplace and Industrial Injury

Workplace injuries account for a significant portion of acquired disability. Construction, agriculture, oil and gas, transportation, manufacturing, and healthcare are among the highest-risk industries. Repetitive strain injuries, chemical exposures, falls, equipment injuries, and motor vehicle collisions on the job all contribute. Some workplace injuries are covered by Workers' Compensation; others are not. People whose disability arose through work and who were not adequately compensated through WCB often end up applying for AISH instead.

ON ACQUIRED DISABILITY

Every category of acquired condition described above shares a common feature: the person who has it was, before the event or diagnosis, indistinguishable from the general population. They had jobs, relationships, plans, and futures that did not include disability. The category did not announce itself in advance. It arrived through a phone call from a doctor, a moment on a highway, an infection, a fall, a workplace incident — and it became permanent. This is the universal pattern. This is the argument the document closes with.

— *Acquired disability — what one Tuesday can do* —

He was forty-three years old and worked in the trades. On a Tuesday in October he fell from a scaffold, sustained a traumatic brain injury and a fracture of his lumbar spine, and was unconscious for three days. He came home from the hospital six weeks later. He could walk, with a cane. He could not return to work — the cognitive effects of the brain injury made the precision and judgment his job required impossible. WCB benefits ran out. He applied for AISH. He was approved on the third application, after extensive specialist documentation. He is now fifty-one. He has not worked in eight years. His marriage ended after five. His savings ended after three. Before October of that year, he had two kids in hockey, a mortgage, and plans for an early retirement at fifty-five. None of those things are still true. His friends, when they see him, sometimes ask when he's getting back to work. He does not know how to explain that 'getting back' is not the right framing. There is no version of him that gets back. There is only the version that exists now, and the version that existed before, and a Tuesday in October that separates them.

SECTION 7: MULTIPLE AND COMORBID CONDITIONS

Disability rarely arrives alone. The 79,290 Albertans on AISH are classified by primary medical condition for administrative purposes, but most have additional conditions across the other diagnostic categories. The clinical literature on comorbidity in disability populations is consistent: multiple conditions are the norm, not the exception.

A few representative patterns:

- **Mental illness with chronic pain.** Estimates of depression prevalence among patients with chronic pain range from 31% to 100%. Pain complaints in depressed individuals range from 34% to 66%. The two conditions amplify each other.
- **Mental illness with substance use disorder.** CAMH documents that at least 20% of people with mental illness have a co-occurring substance use disorder. For people with schizophrenia, the figure rises to as high as 50%, often as a form of self-medication for unrelieved symptoms.
- **Brain injury with mental illness.** Traumatic brain injury substantially increases the risk of subsequent depression, anxiety, PTSD, and substance use disorder. The brain injury itself produces emotional dysregulation that compounds mental health vulnerability.
- **Autoimmune disease with cognitive impairment.** Lupus, multiple sclerosis, and other autoimmune conditions frequently produce cognitive symptoms — sometimes called 'lupus fog' or 'MS fog' — alongside their physical manifestations.
- **Chronic illness with mental illness.** Living with chronic, painful, or disabling physical illness substantially elevates the risk of secondary depression and anxiety. The mental illness is not separate from the physical condition; it is a predictable consequence of it.
- **Developmental disability with mental illness.** Adults with intellectual disability and autism have higher rates of anxiety, depression, and other mental health conditions than the general population — and far less access to appropriate mental health treatment.

WHAT THIS MEANS FOR THE PUBLIC NARRATIVE

Public assumptions about disability often imagine a single, discrete condition — 'the person with the wheelchair,' 'the person with depression.' The clinical reality is closer to: a person whose original diagnosis has produced a cascade of secondary conditions, each interacting with the others, all of which require management at the same time. The 'one condition' framing is administrative shorthand. The lived experience is multi-condition disability — and the supports designed for any one condition almost never address the complete picture.

SECTION 8: LIVING ARRANGEMENTS — THE SPECTRUM OF NEED

Disability does not produce a single living arrangement. The 79,290 Albertans on AISH live across a wide range of housing and support situations, depending on the nature and severity of their conditions, the availability of family support, and the capacity of the provincial system to provide what they need. This section outlines the major categories.

8.1 Independent Living with AISH

Most AISH recipients live independently — in rented apartments, owned homes, with family, in shared housing, or, in some cases, while experiencing homelessness. The AISH Standard Living Allowance (currently up to \$1,901 per month as of 2025) is intended to cover rent, food, utilities, and basic necessities. Recipients living independently manage their own daily activities, banking, shopping, transportation, and medical care, with whatever informal supports are available to them.

The Government of Alberta open data confirms that 86.0% of AISH recipients are single individuals. Most live alone, with a roommate, or with family — not in any institutional setting.

8.2 Home Care Support

Some AISH recipients live in their own homes but require formal support for daily living. Alberta Health Services provides Home Care services for medically eligible recipients — including help with bathing, dressing, medication management, wound care, and household tasks. Home Care is means-tested and capacity-limited; many people who would benefit from it are on waitlists or receive less care than their needs would justify.

Family caregivers provide a substantial portion of in-home support that is not covered by formal Home Care. Adult children caring for aging disabled parents, parents caring for adult disabled children, and spouses caring for partners are all common arrangements. The economic and personal cost to family caregivers is largely uncompensated.

8.3 PDD-Funded Supported Living and Group Homes

Adults with developmental disabilities who qualify for the Persons with Developmental Disabilities (PDD) program may receive funding for various supported living arrangements. PDD does not provide direct cash benefits — instead, it funds services delivered through approved disability service agencies.

- **Family Managed Services** — the adult with developmental disabilities (or their family) directly hires support workers and manages care.
- **Supported Independent Living** — the person lives in their own home or apartment with funded support workers visiting on a defined schedule.
- **Group Home** — the person lives in a residence with several other adults with developmental disabilities, with 24-hour staff support.
- **Host Family / Home Share** — the person lives with a family that has been approved and trained to provide care.

PDD is one of the most fragile programs in the Alberta disability system. Waitlists are long. Many adults with developmental disabilities live with aging parents because no PDD-funded placement is available. The province's

2023 study by the Office of the Ombudsman, 'Denied by Design,' documented systematic failures in PDD intake, eligibility, and service delivery.

8.4 Continuing Care Homes (Type A and Type B)

Some AISH recipients live in approved Continuing Care Homes due to medical needs that exceed what independent living or PDD-funded supported living can manage. Type A Continuing Care Homes (formerly called nursing homes or auxiliary hospitals) provide the highest level of medical and personal care. Type B Continuing Care Homes (formerly called designated supportive living) provide a lower level of medical care with personal support.

AISH recipients in Continuing Care receive a Modified Living Allowance: \$357 per month for personal expenses, plus an accommodation rate (currently up to \$2,286 per month for private rooms in Type A facilities) paid directly to the facility. Health benefits and personal benefits remain available within program rules.

8.5 Community Residences

A small number of AISH recipients (0.1%, or approximately 82 people as of September 2025) live in government-operated Community Residences — facilities for individuals whose specific needs cannot be met in either independent living or standard Continuing Care. These are typically people with severe combined medical and behavioural needs requiring specialized environments.

WHAT TRIGGERS WHICH LEVEL OF SUPPORT

The level of living support a disabled adult requires is determined by clinical assessment — usually documented through specialist reports, occupational therapy evaluations, psychological assessments, and ongoing case management. A person with mild intellectual disability may live entirely independently with informal family support. A person with profound intellectual disability and complex medical needs may require 24-hour group home or Continuing Care placement. The same person's needs can change over time — a person who lives independently at age 30 may need group home placement at age 55 due to age-related decline. Living arrangements in disability are dynamic, not fixed.

SECTION 9: WHAT AISH PROVIDES

The AISH program is described in the Government of Alberta's official guide and the AISH Policy Manual as providing four main categories of support: a monthly living allowance, a monthly child benefit, health benefits, and personal benefits. The actual content of each category, and its limitations, is outlined below.

9.1 Monthly Living Allowance

The maximum AISH living allowance as of 2025 is \$1,901 per month. The actual amount received depends on other income from the recipient and a cohabiting partner, if applicable. Non-exempt income reduces the living allowance dollar-for-dollar above certain thresholds.

The 2024 Maytree report on welfare incomes across Canada documented that the maximum AISH benefit, while the highest provincial disability income in Canada, still leaves recipients substantially below the poverty line. A single AISH recipient in Edmonton at the 2025 maximum receives approximately \$22,812 annually — roughly \$6,000 below the Market Basket Measure poverty line for a single adult.

9.2 Modified Living Allowance (Continuing Care)

AISH recipients in Type A or Type B Continuing Care Homes receive the Modified Living Allowance: \$357 per month for personal expenses, plus the facility accommodation rate paid directly to the facility. This structure ensures the facility is paid while the recipient retains a modest personal allowance for incidentals.

9.3 Monthly Child Benefit

AISH recipients with dependent children receive a monthly child benefit on top of the living allowance. The benefit amount varies by family composition. Child benefits are intended to assist with raising dependent children and are calculated separately from the recipient's own living allowance.

9.4 Health Benefits

AISH recipients receive an AISH Health Benefits Card providing coverage for prescription medications, dental services, optical services, and certain other health-related items. The card is valid only in Alberta. Specific covered items include:

- **Prescription drugs** on the Government of Alberta Drug Benefit List.
- **Some over-the-counter items and nutritional products** as approved by AISH.
- **Basic dental coverage** — including extractions, fillings, and certain routine procedures (specific coverage varies; recipients should consult their dentist).
- **Basic optical coverage** — one eye exam every 2 years for adults; one pair of glasses every 2 years for adults and every year for dependent children.
- **Continuous glucose monitors (CGMs)** for Albertans with diabetes who meet eligibility criteria.
- **Medical equipment and supplies** not available through Aids to Daily Living, private insurance, or other sources.

AISH recipients may also be eligible for the federal Canadian Dental Care Plan (CDCP) starting in 2024–2025. Where both plans cover a service, the CDCP must be billed first.

9.5 Personal Benefits — Three Categories

Personal benefits cover specific needs over and above the monthly living allowance. To be eligible for personal benefits, a recipient must have no more than \$5,000 in non-exempt assets (cash, investments, bonds) or be in a situation of financial hardship. Some personal benefits may need to be repaid. There are three categories:

(1) Health-Related Supports

Help with costs for medical equipment not available through AADL or other sources, medical supplies not covered elsewhere, and certain other health-related items determined on a case-by-case basis.

(2) Childcare and Education

Some costs may be covered for infant and childcare, or for children's education needs.

(3) Emergency Benefits

Help with situations beyond the recipient's control that put the recipient or their dependents at immediate risk. Eligibility is determined case-by-case.

All personal benefits are subject to caseworker review and AISH discretion. A request that fits a benefit category does not guarantee approval.

9.6 Asset and Income Limits

AISH recipients may have personal total net assets of up to \$100,000 in monetary form (savings, investments, bonds) without losing eligibility. The primary residence and one vehicle are exempt from this limit. The asset limit was raised from a much lower threshold over the program's history.

For personal benefits eligibility, the cap is much lower — \$5,000 in non-exempt assets, unless the recipient is in documented financial hardship.

Income from the recipient's employment, self-employment, or partner's employment reduces the AISH benefit on a sliding scale. Some forms of income are partially or fully exempt; others reduce benefits dollar-for-dollar.

WHAT 1.6% OF ALBERTA'S POPULATION RECEIVES

1.6% of Alberta's total population receives AISH. The maximum benefit is \$1,901 per month — below the poverty line. The asset limit for personal benefits is \$5,000. The Health Benefits Card is valid only within Alberta. The provincial program serves the most severely disabled population in the province, and its design implies, every month, that this population's full set of needs can be met within these parameters. The lived experience of the 79,290 Albertans receiving AISH consistently indicates that they cannot.

SECTION 10: WHAT AISH DOES NOT PROVIDE

An understanding of disability supports is incomplete without a clear naming of what those supports do not cover. The list below is not exhaustive. It captures the most common gaps experienced by AISH recipients.

Adequate income. The AISH benefit is below the poverty line in every Alberta city. Recipients living entirely on AISH cannot afford adequate housing, food, transportation, and personal necessities at current costs.

A tax refund. AISH income is non-taxable. Recipients with no other income generate no taxable income and therefore no refund — yet are still required to file taxes annually to maintain eligibility for federal programs and ensure compliance with AISH requirements.

Coverage outside Alberta. The AISH Health Benefits Card is not valid outside the province. Recipients travelling for medical care, family emergencies, or any other reason are not covered for AISH-funded health services while away.

Caseworker continuity. The 2025 transition to a call-centre service model eliminated the previous direct-caseworker relationships. Recipients calling AISH no longer reach a named worker assigned to their file. Each call begins again with a different agent.

Form completion fees. AISH-required medical forms (Application Part B, Disability Assistance Medical Report) are completed by physicians who may charge the recipient directly. There is no regulated maximum fee. The Government of Alberta has committed to covering one assessment cost for current AISH clients transitioning to ADAP, but historic and ongoing form fees remain the recipient's responsibility.

Sufficient mental health support. While AISH covers prescriptions on the formulary, it does not cover most psychotherapy, psychological assessments not required for the program itself, or specialized mental health services not available through AHS. Recipients with mental illness as their primary diagnosis often face mental health treatment gaps the program does not address.

Adequate dental and optical coverage. Basic dental and optical coverage is limited in scope. Major dental work, specialty optical needs, and many treatments are not covered or are covered at inadequate levels.

Coverage for paid tax preparation. Recipients who cannot navigate tax filing themselves and must pay for assistance bear that cost out of pocket from a benefit that produces no refund.

Equity for parents. FSCD (for children under 18) and PDD (for developmentally disabled adults) are separate programs from AISH. Parents of disabled children navigating multiple programs simultaneously face cumulative administrative burden that none of the programs individually accounts for.

Adequate housing supports. AISH does not provide subsidized housing directly. Recipients seeking subsidized or supported housing must apply through separate provincial and municipal programs with their own waitlists, eligibility criteria, and processing times.

Transportation. AISH does not cover transportation costs. Many recipients rely on subsidized transit passes (administered through municipal programs) or have no reliable transportation at all.

Recovery from administrative error. When AISH or a federal partner agency makes an administrative error producing a benefit reduction or hold, the recipient bears the burden of identifying it, documenting it, and pursuing correction. There is no automatic accountability mechanism.

Independent appeal rights for ADAP eligibility decisions. Bill 12 (December 2025) removed independent appeal rights for AISH and the new ADAP program. Eligibility decisions and reassessment outcomes can no longer be appealed through an independent tribunal as they previously could.

WHAT THE GAPS ADD UP TO

The list above does not represent oversights or growing pains in an otherwise functional system. It represents structural limitations of the AISH program as designed. Each gap places real burden on recipients whose conditions are precisely the conditions making the burden hardest to bear. The cumulative effect is a program that, on paper, provides comprehensive support — and, in lived experience, leaves substantial portions of the support burden to recipients, family, charity, or nothing at all.

CLOSING ARGUMENT: ONE ACCIDENT. ONE DIAGNOSIS.

Every disabled person reading this document already knows what comes next. The sections above describe lives they live, conditions they navigate, and gaps they fall into. They do not need to be persuaded. The closing argument is for the readers who are not disabled — yet.

The argument is short. The data is substantial. Public skepticism about disability is, ultimately, an artifact of a particular kind of denial: the conviction that the line between 'people who need disability supports' and 'people who do not' is fixed, identifiable, and unlikely to be crossed by the person doing the convincing. The conviction is wrong on every empirical measure available.

How Likely Is It That You Will Need These Supports?

Statistics Canada (2022) reports that 27% of Canadians aged 15 or older — approximately 8 million people — have a disability that limits their daily activities. The figure has risen steadily over the past decade. By age 65 and older, the disability rate exceeds 40%. By age 75, it exceeds 50%.

These figures are not a forecast of unfortunate accidents. They are the baseline rate of human aging and human exposure to chronic illness. The expected outcome of a long life is, eventually, disability.

The Categories of Risk

- **Accident.** Approximately 165,000 Canadians sustain a brain injury each year. Approximately 4,300 sustain a new spinal cord injury. Tens of thousands experience workplace, vehicle, sports, and home injuries that produce permanent disability. None of these events are predicted in advance by the people they happen to.
- **Illness.** Approximately 1 in 2 Canadians develops cancer. Approximately 1 in 3 develops cardiovascular disease. Approximately 1 in 5 develops chronic pain. Approximately 1 in 10 develops a major mood disorder. These are not low-probability events. They are the standard diseases of human populations.
- **Infection.** Long COVID has produced an estimated millions of new cases of post-viral disability in Canada alone since 2020. Other post-viral and post-bacterial conditions (post-Lyme, post-mononucleosis, post-sepsis) have produced disability for decades. A single infection in a person previously healthy can produce permanent disability.
- **Aging.** Cognitive decline, sensory loss, mobility impairment, and chronic illness are predictable consequences of human longevity. The percentage of Canadians with disability rises monotonically with age past forty. The graph does not bend back down.
- **Genetic timing.** Some genetic conditions express in childhood (Down syndrome, cerebral palsy, cystic fibrosis). Others do not express until adulthood (Huntington's disease, some forms of muscular dystrophy, hereditary cancers). The person who is well at thirty may have been carrying a condition all along that becomes apparent at forty-five.
- **Workplace exposure.** Decades of occupational exposure to repetitive strain, chemicals, noise, vibration, and physical stress produce disability that often does not appear until later in a working life. Construction, agriculture, healthcare, and industrial workers carry elevated lifetime risk.

What This Means

It means that disability is not, as a rule, the condition of people who lived differently from you. It is, much more often, the condition of people who lived almost identically to you, until something happened. The something might

be a fall on an icy sidewalk. A diagnosis at a routine checkup. A car accident on the way home from work. An infection that did not resolve. A genetic condition that announced itself at forty-six. The arrival of a chronic pain that started as a backache and never stopped.

The support systems described in this document — AISH, PDD, FSCD, the federal disability programs, the entire architecture of Canadian disability policy — exist because human bodies and brains are vulnerable in predictable ways, and a society that does not catch the people who fall is a society that will, eventually, fail to catch you.

The current Alberta government has been narrowing these supports. The ADAP transition scheduled for July 1, 2026 will reassess approximately 79,290 disabled Albertans against criteria the government has not yet fully published. Bill 12 has removed independent appeal rights. The federal Canada Disability Benefit, designed to lift disabled Canadians out of poverty, is being clawed back dollar-for-dollar by Alberta — the only province in Canada doing this. The cumulative effect of these changes is to reduce, by attrition and design, the safety net described in the preceding pages.

THE REAL POSITION TO HOLD

If you are reading this document and you do not currently have a disability, the question is not whether the disabled population deserves the supports the system provides. The question is whether you trust the system to be there when your turn comes. Every disabled person in this province is, at the most basic level, a person who already entered a category that did not exist in their lives before. The system that did or did not catch them is the same system that will or will not catch you. Public indifference to its erosion is not a position about other people. It is a bet on your own future. The historical record is clear on how this bet usually pays out.

The Closing Statement

It takes one accident.

One diagnosis.

Disability does not discriminate.

It does not check your tax bracket. It does not consult your work ethic. It does not consider whether you have been a good person, a careful person, a deserving person. It does not arrive at predictable times in predictable lives. It arrives in the lives of construction workers, doctors, schoolteachers, lawyers, mechanics, mothers, fathers, athletes, accountants, retirees, and university students. It arrives in the lives of people who voted Conservative and people who voted NDP. It arrives in the lives of people who attend church and people who do not. It arrives in the lives of immigrants and people whose families have lived in Canada for two centuries. It arrives in the lives of people who exercised their entire lives and people who never did. It arrives in the lives of people who 'always took care of themselves' and people who, for whatever reason, did not.

It will, eventually, arrive in yours. Or in the life of someone you love. The supports described in this document — the imperfect, underfunded, eroding, but still-existing supports — are the supports that catch the people it has already arrived for. They are the supports that may, one day, catch you.

The Albertans on AISH are not other people. They are us, on a different timeline, after a different event, holding a different diagnosis. What we owe them is not charity. What we owe them is recognition — that the system serving them is the system that will serve us, and that what is being done to it now is being done to us all.

The Alberta Disability System Breakdown

This document was compiled to make visible what is too often left to disabled Albertans alone to explain. It is published in the conviction that public education is the precondition for political change, and that the conditions described here cannot continue under serious public scrutiny.

albertadisabilitysystembreakdown.netlify.app

SOURCES AND REFERENCES

Government of Alberta — Primary Documents

- Government of Alberta, Assisted Living and Social Services — AISH Caseload Open Data, September 2025. Primary diagnosis breakdown: Physical Disabilities 32,901 (41.5%); Mental Illness Disorders 23,940 (30.2%); Cognitive Disorders 22,419 (28.3%); Unknown 30 (0.0%). Total 79,290.
- Government of Alberta — Your Guide to AISH (April 2024 / updated July 2024 / May 2025 publication). Comprehensive program overview including living allowance, modified living allowance, health benefits, and personal benefits structures.
- Government of Alberta — AISH Policy Manual. Modified Living Allowance (\$357 personal allowance, \$2,286 private room Type A accommodation rate).
- Government of Alberta — Persons with Developmental Disabilities (PDD) Eligibility documentation. IQ ≤ 70 , developmental disability before age 18, six or more of 24 daily living skills.
- Government of Alberta — alberta.ca/aish-what-you-get. Personal benefits eligibility (\$5,000 non-exempt assets); three personal benefit categories.
- Government of Alberta — Bill 12 (December 9, 2025). Removal of independent appeal rights for AISH and ADAP eligibility decisions.
- Office of the Alberta Ombudsman — 'Denied by Design' (2024 PDD report). Documented systematic failures in PDD intake and service delivery.

Government of Canada — Primary Documents

- Statistics Canada — Canadian Survey on Disability (CSD), 2017 and 2022 cycles. Disability prevalence (27% of Canadians 15+); developmental disabilities (1.5%, ~456,630 individuals); disability type breakdowns.
- Public Health Agency of Canada — Infographic: Developmental Disabilities or Disorders in Canada (2017 CSD highlights). 5.1% of Canadians with a disability report a developmental disability or disorder.
- Government of Canada — Annex: Understanding Disabilities. Accessible Canada Regulations Guidance. Mobility, mental health, developmental, and other disability category descriptions.
- Government of Canada Digital Accessibility Toolkit — Cognitive Disabilities (a11y.canada.ca). Definitions of intellectual disability, autism, dyslexia, dyscalculia.
- Statistics Canada — Mental Health-Related Disabilities in Canada, 2017. Mental health disability category definitions and prevalence.

Clinical and Research Sources

- Centre for Addiction and Mental Health (CAMH) — Mental Illness and Addiction: Facts and Statistics. Substance use comorbidity (20% of mental illness, 50% of schizophrenia); mental illness as a leading cause of disability.
- Statista — Mental Health Issues in Canada, Statistics & Facts. Mood disorder prevalence (over 11% of Canadians).
- Institute for Health Metrics and Evaluation — Global Burden of Disease, Canada 2017. Mental disorders accounted for 10.6% of DALYs.
- U.S. Federal Communications Commission — Cognitive Disabilities. Definition: intellectual disability, autism, severe persistent mental illness, brain injury, stroke, dementias.

- World Wide Web Consortium (W3C) — Web Accessibility Initiative. Cognitive and Learning Disabilities. Functional definitions and invisibility characteristics.
- Brain Injury Association of America — Brain Injury as a Form of Invisible Disability.
- Brain Injury Canada — annual incidence statistics (~165,000 brain injuries per year).
- Canadian Spinal Research Organization — spinal cord injury prevalence and incidence.
- Canadian Pain Society — chronic pain definition and prevalence (~8 million Canadians).
- Government of Canada — Action Plan for Pain in Canada.
- Multiple Sclerosis Society of Canada — MS prevalence in Canada (highest national rate worldwide).
- ME Action Network and Canadian ME/CFS clinical resources — post-exertional malaise, severity ranges, long COVID overlap.
- Canadian Cancer Society — lifetime cancer risk (~1 in 2 Canadians).
- American Association of Intellectual and Developmental Disabilities (AAIDD) — three-criterion definition of intellectual disability.

Disability-Specific Resources

- Inclusion Alberta — Canada Disability Benefit Fact Sheet, July 2025; school inclusion advocacy resources.
- Inclusion Canada — national developmental disability advocacy resources.
- AIDE Canada — Autism funding and programs for adults 18+ in Alberta.
- Disability Action Hall (Alberta, 28 years of organizing prior to 2024 funding cut) — historical advocacy resources.
- Canadian Mental Health Association — mental illness as disability resources and population statistics.
- Disabled World — Cognitive Disability and Intellectual Disability overviews.
- Maytree Foundation — Welfare Incomes Across Canada, 2024. AISH benefit analysis relative to poverty line.

Companion Documents from The Alberta Disability System Breakdown Archive

- The Access Gap: Penalized for Non-Ability (May 2026). Companion analysis of how Alberta and federal disability administrative systems are inaccessible to the populations they were designed to serve.
- Information Asymmetry Report. Coverage of what is not communicated by the program to recipients.
- AISH Evidentiary Report Series, April 2026. Primary-source documentation of compliance, communication, and access barriers.
- Healthcare Worker Shortage Report. 650,000 Albertans without a family doctor.
- Compound Failure Brief. Synthesis of AISH, ADAP, FSCD, PDD, healthcare access, and appeal rights as a single failing system.
- All companion documents available at albertadisabilitysystembreakdown.netlify.app — free to share, free to print, free to distribute.

ABOUT THIS REPORT

This document was compiled by The Alberta Disability System Breakdown — a community-led documentation campaign opposing the AISH-to-ADAP transition, FSCD/PDD waitlists, healthcare access failures, the federal Canada Disability Benefit clawback, and the structural inaccessibility of Alberta and federal disability systems. It is written as a public education resource for use by advocates, families, journalists, elected officials, and members of the general public who want to understand what the conditions named in disability policy actually mean. All figures are sourced from Government of Alberta open data, Statistics Canada, peer-reviewed clinical research, and authoritative disability advocacy organizations. Published May 2026. Free to share. Free to print. Free to distribute. albertadisabilitybreakdown@outlook.com | albertadisabilitysystembreakdown.netlify.app